

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$20,655,771
Prenatal arm	\$9,897,557
Postnatal arm	\$10,758,214
Administrative load (8%)	\$1,652,462
Appropriation total (benefit + admin)	\$22,308,233
Sampling 90% CI	\$15,698,769 - \$25,612,773
Assumption band	\$12,393,463 - \$26,336,706

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) - 3-mo postnatal	\$3,000	\$20,655,771	\$22,308,233	13,771
Statutory (Medicaid OR <=300% FPL) · 6-mo postnat	\$4,500	\$31,413,986	\$33,927,104	13,771
Universal · 6-mo postnatal	\$4,500	\$52,145,328	\$56,316,954	22,858
Universal · 12-mo postnatal	\$7,500	\$87,861,306	\$94,890,211	22,858

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$32,274,643
Total cost (12-month design)	\$52,930,414

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

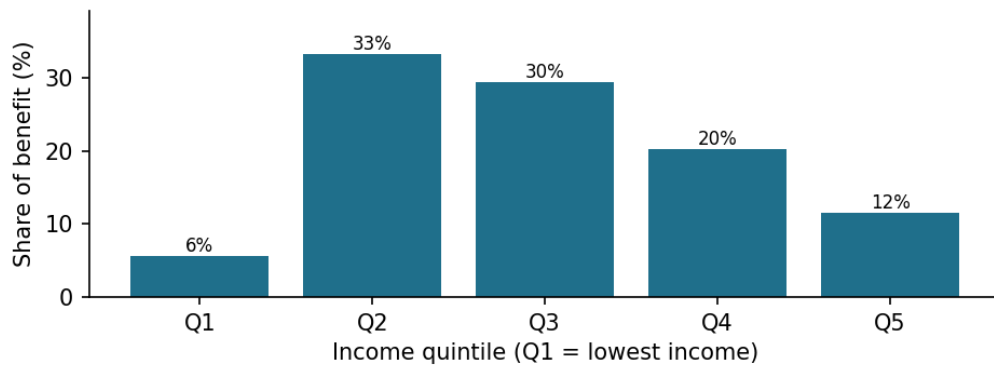
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$10,316,928
Prenatal / Postnatal	\$5,361,177 / \$4,955,751
Postnatal deferred to next FY	\$871,615
Year-1 as % of steady state	50%
Ramp sensitivity (6 / 12 / 18 mo)	\$15,455,968 / \$10,316,928 / \$6,877,952

Household reach

Births served / year (people affected)	7,969
Eligible families (weighted)	7,340
Expected recipients / year	13,771
Expected pregnancies / infants	6,598 / 7,172
Avg benefit per recipient	\$1,500

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	761	\$1,140,991	5.5%
Q2	\$57,024	110,263	4,591	\$6,886,144	33.3%
Q3	\$96,722	110,317	4,059	\$6,088,915	29.5%
Q4	\$154,809	110,231	2,779	\$4,168,676	20.2%
Q5	\$334,084	110,343	1,581	\$2,371,046	11.5%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$4,466,824	\$2,140,353	\$2,326,471	2,978
Honolulu	\$13,643,551	\$6,537,535	\$7,106,016	9,096
Kauai	\$1,474,678	\$706,616	\$768,061	983
Maui	\$1,070,719	\$513,053	\$557,666	714

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.